

DISCHARGE SUMMARY

Patient	Ravi Kumar	Age/Sex	58 Years / Male
IP No.	APL-2024-08741	Ward	Cardiology — Room 412
Admitted	12-Nov-2024	Discharged	19-Nov-2024
Consultant	Dr. Kavitha Menon, DM Cardiology	Dept.	Cardiology

PRIMARY DIAGNOSIS

- Type 2 Diabetes Mellitus — poorly controlled (HbA1c 9.2%)
- Hypertensive Heart Disease with Left Ventricular Hypertrophy
- Dyslipidaemia — mixed hyperlipidaemia
- Stable Angina Pectoris — NYHA Class II

HISTORY

58-year-old male, known T2DM since 2016 and Hypertension since 2018, presented with chest tightness and exertional dyspnoea for 3 days. Mild pedal oedema. Non-smoker, non-alcoholic. Retired schoolteacher from Chennai.

SIGNIFICANT PAST HISTORY

- T2DM since 2016 — on Metformin 1g BD, Glimepiride 2mg OD
- Hypertension since 2018 — on Amlodipine 5mg OD, Telmisartan 40mg OD
- Dyslipidaemia since 2020 — on Atorvastatin 40mg OD

ALLERGY: Penicillin — documented anaphylaxis 2019. AVOID ALL PENICILLINS.

INVESTIGATIONS

Test	Result	Reference	Status
HbA1c	9.2%	< 7.0%	HIGH ↑
Fasting Blood Sugar	214 mg/dL	70–100	HIGH ↑
Serum Creatinine	1.3 mg/dL	0.7–1.2	BORDERLINE
eGFR	62 mL/min	> 60	CKD Stage 2
LDL Cholesterol	142 mg/dL	< 100	HIGH ↑
HDL Cholesterol	38 mg/dL	> 40	LOW ↓
Troponin I	0.02 ng/mL	< 0.04	NORMAL
Echo 2D	EF 52%, LVH, Grade 1 diastolic dysfunction		—

DISCHARGE MEDICATIONS

Medication	Dose	Frequency
Metformin	1g	Twice daily
Glimepiride	2mg	Once daily
Amlodipine	5mg	Once daily

Telmisartan	40mg	Once daily
Bisoprolol	2.5mg	Once daily (NEW — added this admission)
Rosuvastatin	20mg	Once at night (switched from Atorvastatin)
Aspirin	75mg	Once daily
Isosorbide Mononitrate	30mg	Once daily — 3 months

FOLLOW-UP

- Cardiology OPD in 4 weeks — repeat Echo and ECG
- Endocrinology review in 6 weeks — target HbA1c < 7.5%
- Repeat HbA1c, Lipid profile, Renal function in 3 months
- Low-sodium, low-GI diet. 30 mins walking daily. BP target < 130/80 mmHg